

إسم المريض ربيعاً : عنايات صابر عطيه ابراهيم
المحافظة : الجيزه
رقم البطاقة : 01156960873 رقم التليفون : 26401240102041
العمر(تاريخ الميلاد) : 24-01-1964 المهنة : بدون عمل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

**رئيس
المجلس الطبي المتخصص**

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Name: عنايات صابر عطيه ابراهيم National ID: 26401240102041	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

عضيات
ص. ابر عطية ابراهيم
١٠٢ ش عرفة - البراجيل
مركز اوسيم - الجيزة

١٩٦٤/٠٢/٢٤ ٢٦٤٠١٢٤٠١٠٢٠٤١

IU9097779

١٥ م
سونا رعد العدة المرقية

٢٠٢١/٠٣ ٢٦٤٠١٢٤٠١٠٢٠٤١

ربة منزل
أنثى
مصلحة
ارملة

البطاقة سارية حتى ٢٠٢٨/٠٢/١٧

Patient Name : الأحمدة / علياء صابر عطية الزايد
Patient No. : 26401240102041
Path No. : 2241
Sex : Female
Age : 57 Y
Referring Doctor:

Request No.	: 136723
Request Date	: 26/09/2021
Verification Date	: 10/10/2021
Financial Category	: اورام نادر السلام - حبيتر
Department	: Out Patient
IHC No.	:

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Right breast mass

Nature of the specimen:

Tru-Cut biopsy

Gross Examination:

5 greyish white and hemorrhagic tissue cores, up to 1.5cm. Totally submitted.

Microscopic examination:

- Histological type: Papillary ductal carcinoma insitu with focal microinvasion
- Resection Margins: N/A
- Histologic response to therapy: N/A
- Additional pathologic findings: N/A

III

P63: Negative

Calponin: Negative

ER: Diffusely positive.

Diagnosis:

Right breast mass; Tru-Cut biopsy:

Papillary ductal carcinoma insitu with focal microinvasion

Extent of invasive tumour to be re-assessed on re-biopsy or tumour excision specimen.

Prof. Dr. Iman Gouda

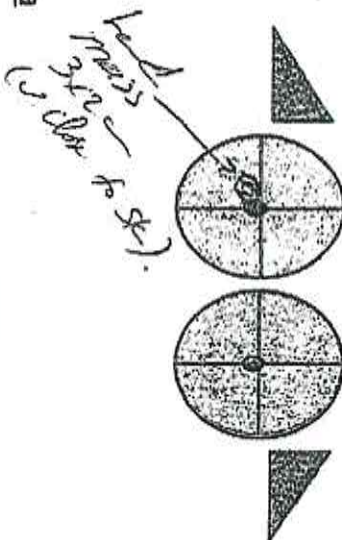
Dr. Abdulrahman Ibrahim

مملكة العربية السعودية
وزارة الصحة العامة
السرطانالسرطان
مملكة
السعودية

a.e1111/5 MDT FORM

Name	أحمد بن محمد
Age	35
MRN	1000000000
ID	1000000000
Co morbidity	115V53Y3N1

Clinical Data



Bx:	Pancreatic Cancer	SM:	Tumor
IHC:	+ invasive front	Mets Workup:	CTAP -> hepatic + v. helix Trophus CT -> hepatic cyst Bone scan -> free
ER:			
PR:			
Hert:			
KIV:			
T N M			

COMMENT:

① For management advised
to take from pathology for
independent L.N.
MDT OUTCOME & PLAN
Men re-MDT.

MDT
report up to T. 11/11/15
for report re. CLE (card quadrants) + SIBS
21/12/2015
21/12/2015

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي
Presidential Initiative for Women's Health - Pathology Lab

Patient Name	: الاستاذة / غلايت صليح عطية ابراهيم
Patient No.	: 26401240102041
Path No.	: 14/2022
Sex	: Female
Age	: 57 Y
Referring Doc.	:
Request No.	: 135749
Request Date	: 30/12/2021
Verification Date	: 13/01/2022
Financial Category	: اورام دار السلام - مبداء
Department	: Out Patient
IHC No.	:
Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

CLINICAL DATA

Right breast papillary lesion.

NATURE OF SPECIMEN

Right wide local excision & sentinel lymph nodes.

GROSS PATHOLOGY

Two specimens were received;

- 1- Right WLE specimen: measured 14x12x3 cm. Skin ellipse: 13x7 cm. Cut section revealed vaguely defined nodular grayish white and hemorrhagic masses; 3x2.5x2 cm, located just below the skin.
- 2- Right Sentinel lymph nodes: Fibrofatty tissue piece; 2x1 cm, entangling five nodules measured up to 2 cm in diameter.

Procedure: Wide local excision.

Localization: Right.

Tumor extent

Number:1

Size: 3x2.5x2 cm

- Skin involvement: just below the skin

Distance from margins: free margins

Lymph nodes number and size: 5 nodules measured up to 2 cm in diameter.

Fluorescence section: Free resection margins & free sentinel lymph nodes (0/5).

Microscopic examination:

Histological type: Encapsulated papillary carcinoma.

Grade (differentiation + nuclear pleomorphism + mitosis): NA

Angioinvasion: No

Ductal carcinoma in-situ: This Tumor is considered as DCIS according to WHO classification 2019.

Verified by:

Dr. Marwa mahmoud

Printing Date : 23/01/2022 10:32:06

Page 1 of 2

Printed By : Ali mostafa



**POST CONTRAST MULTISLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS
REVEALED:-**

**** Known case of breast cancer**

1. Chest:

- Both breasts hang out of the scanned region showing:
 - Suspected right breast parenchymal mass at the edge of the scanned area.
 - Enlarged right supraclavicular LN measuring 1.2 x 0.8 cm.
- Middle lobar (lateral segment) tiny pulmonary nodule measuring 5x 3 mm is seen.
- Otherwise, No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Small hiatus hernia is noted.
- Enlarged left thyroid lobe with retro-sternal extension.

2. Abdomen and Pelvis:

- The liver is enlarged. Right hepatic lobe(segment VI) hypodense focal lesion is seen measuring 1.3 x 0.8 cm.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- underfilled urinary bladder with no stones.
- Bulky uterus.
- No abdominal or pelvic lymphadenopathy or collections.
- Few dorsal vertebral lytic lesions with vertical striations, largest at D4, likely hemangiomas.

OPINION:

- Both breasts hang out of the scanned region showing:
 - Suspected right breast parenchymal mass at the edge of the scanned area.
 - For correlation with sono-mammographic results.
- Middle lobar (lateral segment) tiny pulmonary nodule, likely insignificant, for follow up.
- Right hepatic lobe(segment VI) hypodense focal lesion for US correlation.
- Few dorsal vertebral lytic lesions with vertical striations, largest at D4, likely hemangiomas.

Dr. Peter Nashaat, MSc

Dr. Amr Elgouh MD



الجمهورية العربية السورية
الوزارة العامة للصحة

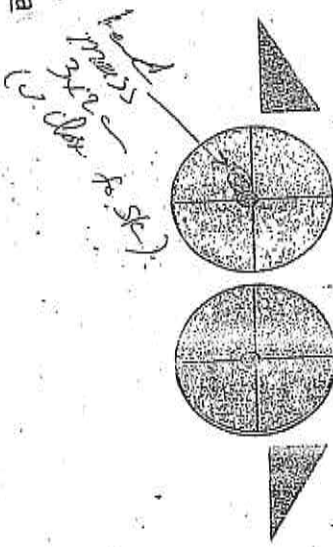
الوزارة العامة للصحة
100

a. el 111/5 MDT FORM

2021
a. el 111/5

Name	Elmer Elmer
Age	25
MRN	1000000000
ID	1000000000
Co	1000000000
morbidity	1000000000

Clinical Data



Bx:	Primary breast carcinoma	SM:	Tumor
IHC:	ER: +	PR: +	HER2: +
PR:	ER: +	PR: +	HER2: +
HER2:	ER: +	PR: +	HER2: +
KTV:	ER: +	PR: +	HER2: +

COMMENT:

① For metastatic disease
② For breast carcinoma
③ For breast carcinoma
④ For breast carcinoma
⑤ For breast carcinoma
⑥ For breast carcinoma
⑦ For breast carcinoma
⑧ For breast carcinoma
⑨ For breast carcinoma
⑩ For breast carcinoma

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN	MDT OUTCOME & PLAN	MDT OUTCOME & PLAN	MDT OUTCOME & PLAN
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MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
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ca 111/5

Patient Name : الأستاذة / غدايات صابر عطية ابراهيم
Patient No. : 26401240102041
Path No. : 14/2022
Sex : Female
Age : 57 Y
Referring Doc. :
Request No. : 135749
Request Date : 30/12/2021
Verification Date : 13/01/2022
Financial Category : أورام دار السلام - مبادره
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

CLINICAL DATA

Right breast papillary lesion.

NATURE OF SPECIMEN

Right wide local excision & sentinel lymph nodes.

GROSS PATHOLOGY

No specimens were received;

Right WLE specimen; measured 14x12x3 cm. Skin ellipse; 13x7 cm. Cut section revealed vaguely defined nodular grayish white and hemorrhagic masses; 3x2.5x2 cm. located just below the skin. Right Sentinel lymph nodes: Fibrofatty tissue piece; 2x1 cm. entangling five nodules measured up to 2 cm in diameter.

Procedure: Wide local excision.

Localization: Right.

Tumor extent

Number: 1

Size: 3x2.5x2 cm

Skin involvement: just below the skin

Distance from margins: free margins

Lymph nodes number and size: 5 nodules measured up to 2 cm in diameter.

Cross section: Free resection margins & free sentinel lymph nodes (0/5).

Microscopic examination:

Histological type: Encapsulated papillary carcinoma.

Grade (differentiation): nuclear pleomorphism + mitosis: NA

Stromal invasion: No

Final carcinoma in-situ: This Tumor is considered as DCIS according to WHO classification 2019.

Verified by :

Dr.Marwa mahmoud

Printing Date : 20/01/2022 10:32:06

Page 1 of 2

Printed By : Ali mostafa

Patient's Name: Inayat Saber Atieh Ibrahim
Date: 2026-08-29
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

NECK ULTRASONOGRAPHY:

- Status post right thyroidectomy
- No evidence of recurrence, residual or lesions.
- The left thyroid gland enlarged with multiple isoechoic nodules largest measuring 2.7x2.5cm.
- Sub-mandibular glands in both sides are of average size with normal texture, no focal lesions.
- Parotid glands in both sides are of average size with normal texture, no focal lesions.
- No pathologically enlarged cervical lymph nodes.

CONCLUSION:

- Left thyroid nodule (TIRAD 4) for FNA.

Thanks for reference
Yours

Dr. Sahl Mahmoud



Name : ANAYAT SABER

Date : 26-01-2026 05:37

Branch : Portable

Medical No. : GRI-260126173659

Ref.Physician Prof :

Ins.Company :

Name: Enayat saber

Date: 26 January 2026

Neck ultrasound examination

Findings

Thyroid gland

- Right hemithyroidectomy
- Enlarged left thyroid lobe by virtue of multiple solid hyperechoic nodules, the largest is measuring 25x21mm& showing

Composition Solid 2

Echotexture Hyperechoic 1

Margin Smooth 0

Shape Wider than taller 0

Calcifications None 0

Score 3 (TIRADS 3)

- No gross retrosternal extension
- Normal movement with swallowing
- The area of patient concern at the left mandible display sub centimetric LN measuring 9x4mm likely no specific& another few sub centimetric cervical lymph nodes, all are oval shaped with preserved hila likely non specific
- Other neck structures;
 - ☐ Trachea is centralized
 - ☐ Normal neck vasculature
 - ☐ Normal mobility of vocal cord o phonation



Name : ANAYAT SABER

Date : 26-01-2026 05:37

Branch : Portable

Medical No.: : GRI-260126173659

Ref.Physician Prof :

Ins.Company :

Opinion

Multiple left thyroid lobe nodules (TIRADS 3)

RECOMMENDATION

FNAC of the largest one is recommended. (TRADS 3, FNAC if > 2.5 cm follow if <2.5cm)

Best regards

Dr. NERMEEN RAMADAN FERR, MD

DOC GRI

DOC GRI